

Transformative or overhyped? The impact of weight-loss drugs on European food demand

The global market for GLP-1s will reach \$100bn in 2027. Yet, the impact of these drugs on food demand is still limited in Europe. Around 2% of European adults are using GLP-1s, but that number will grow. Food makers face a gradual demand shift, giving them time to respond by changing products and marketing and investing in markets with less GLP-1 use



The move towards oral GLP-1s could reshape demand in the years ahead

Obesity and Type 2 diabetes are significant societal and economic problems

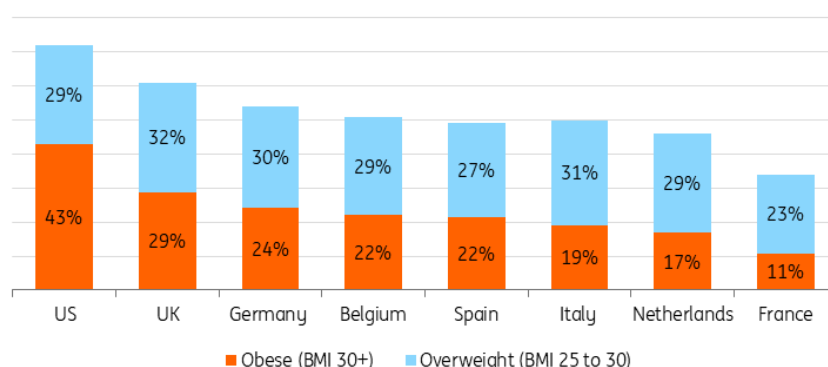
Obesity rates have risen sharply over the past few decades: in 1975, a little over 10% of US adults were obese; in 2022, 43% of US adults were obese. In Europe, obesity rates are lower but still considerable. In fact, there are currently more people overweight than malnourished worldwide. Weight loss drugs (or GLP-1s) are very effective in both the treatment of diabetes and for weight reduction (users typically lose 12-22% of body weight). No wonder everyone is looking at these

blockbuster drugs as a powerful tool to reverse rising obesity rates.

This is not just important because it helps people lead healthier lives, but obesity also carries high societal costs. It is linked to costly health issues such as cardiovascular diseases, joint problems and diabetes. Additionally, obesity reduces labour force participation and lowers productivity, as well as impacting physical and mental well-being.

Obesity rates are lower in Europe than in the US and vary widely within Europe

Share of adults, 2022

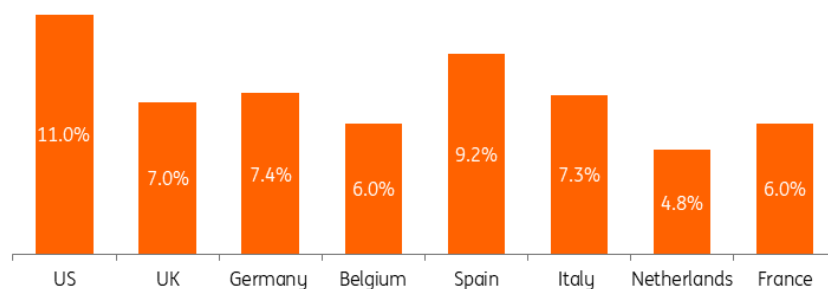


Source: World Health Organization, Global Health Observatory

Prolonged obesity can increase the risk of diabetes significantly. Roughly 90% of all diabetes patients have Type 2 diabetes, which means that they develop diabetes over their lifetime rather than being born with it. Genetics, lifestyle, and weight are key factors in developing Type 2 diabetes. This makes the case for GLP-1s even stronger, because diabetes takes a significant toll on patients and is a chronic disease with very costly complications (such as kidney dialysis or cardiovascular diseases).

Type 2 diabetes is a common chronic disease in Europe and the US

Share of adults with Type 2 diabetes, 2024



Source: International Diabetes Federation, ING Research

Use of weight-loss drugs is growing quickly

With that in mind, it's no surprise that GLP-1s have seen a very steep growth trajectory since they were introduced. In 2025, the global market size for GLP-1s was roughly \$70bn, which includes treatment for diabetes and weight loss. Roughly one in five US adults has used a weight-loss drug, and current use is estimated to be around 12%.

In Europe and the UK, we estimate that at least 2% of adults are currently using them, which means about nine to 10 million users. Our estimate considers a variety of sources, including data from public health authorities, consumer surveys and revenue data from Novo Nordisk and Eli Lilly. Still, it's likely to be conservative, as actual usage precedes hard data.

In Europe, weight-loss drugs have been adopted more quickly in wealthier countries like the UK and the Nordics. Central and Eastern European countries show slower uptake even though they're among the countries with the highest obesity rates. There is a great variety of users. You'll have people who pay out-of-pocket to use them for weight management and people who get them as a treatment for Type 2 diabetes. In terms of age and gender, older consumers and women are over-represented.

A lot of people don't stick to the drugs. Studies show that 50% discontinue GLP-1s within one year for various reasons, including costs, side effects or simply because they've reached a certain target weight. That is important for food and beverage companies that could see some of their products falling temporarily in and out of favour with consumers as a result.

12% vs
2%

Use of GLP-1s amongst
adults

US vs EU+UK

Market size to reach \$100bn in 2027

The trillion-dollar question is: how will demand evolve over the next few years? As mentioned, the market for GLP-1s was \$70bn last year. Several factors will determine the future market size.

First, the GLP-1 market will become predominantly oral in the US in 2026 and in Europe in 2027. This means lower prices (up to 50%) and likely higher use. Second, more companies will launch their own weight loss drugs, which means more competition and price pressure. Third, governments will consider reimbursing costs for more patients if weight loss is sustainable and helps with prevention, which would further increase demand. Considering that revenues doubled for many weight-loss drugs in 2025, the size of the problem and the fact that only 40% of people living with diabetes receive treatment (Novo Nordisk), we estimate that the market will grow by 20% in the next two years. That means it would hit \$100bn by the end of 2027.

Yet, this still leaves the biggest question unanswered: how sustainable is the weight loss achieved

with GLP-1s? Current [academic research](#) indicates that patients gradually regain weight when they stop using GLP-1s. It is important to note that this often happens with people who achieve significant weight loss, as obesity is a complex disease. The final verdict has not been cast, especially as newer therapies are being developed and the use of GLP-1s for weight loss is still in its infancy in Europe.

Very limited impact on food and beverage demand in 2026

Many argue that GLP-1s can have a transformative impact on aspects of modern life, including our food consumption habits. Questions on the impact do pop up during earnings calls of major food makers, but the drugs are not viewed as an immediate threat. That makes sense; different studies indicate that GLP-1 users reduce their calorie intake by 15-20%. So, if 2% of all adults in the EU and UK use them, then the current impact on total calorie demand is about 0.25%. That's not a worrying number for food and beverage companies yet.

While GLP-1 use is growing fast, it's happening from a low base, so we don't anticipate a massive blow to demand in 2026 and 2027. Still, it shouldn't be neglected. There is already little volume growth in the food market in Europe. Studies in the US point out that expenditure and intake of categories like savoury snacks, candy, chocolate and alcoholic beverages are more exposed (see [here](#) and [here](#)).

So, if GLP-1 adoption goes up, it can become a drag on growth potential for companies operating in affected categories. In response to changes in demand, bakery company Grupo Bimbo has put more emphasis on reformulation, portion control and sugar reduction, while The Magnum Ice Cream Company has detected a shift to fewer but more premium treats. Eventually, changes in consumption also trickle down to ingredient suppliers. As an example, some sugar traders have reduced their [sugar consumption figures](#) for North America and Europe as a result.

Households cut spending on most food and beverage items when a member uses GLP-1s

Changes in grocery spending of US households on a selection of categories, 6 months post GLP-1 adoption

Change in grocery spending	Category
>10% reduction	Chips & savoury snacks
5-10% reduction	Sweet bakery, cheese, meat
2.5-5% reduction	Fresh vegetables, soft drinks, milk, coffee/tea/energy drinks, candy & chocolate
0-2.5% reduction	Alcohol
0-2.5% increase	Fresh fruit

Source: Hristakeva, Sylvia and Liaukonyte, Jura and Feler, Leo, The No-Hunger Games: How GLP-1 Medication Adoption is Changing Consumer Food Demand.

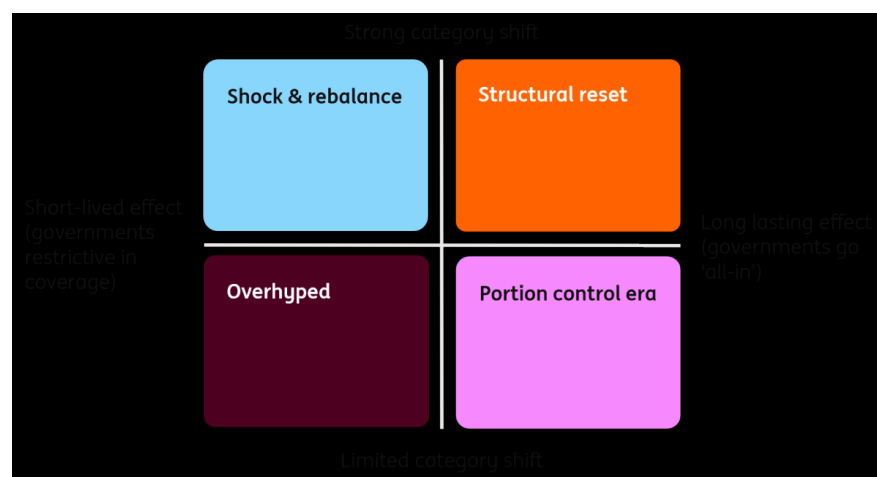
Transformative or overhyped? The potential impact by 2030

Despite increasing use and their possible economic benefits, much is still uncertain about the longer-term impact of weight-loss drugs on the food and beverage market. To provide senior executives with a framework, we propose four scenarios in which we differentiate between our two biggest questions. The first is the duration of the effect on weight, calorie intake and body weight.

If there is a sustained long-term effect, we expect that governments will be more inclined to rapidly expand coverage. If not, then governments remain restrictive.

The second dimension is the strength of any shifts between categories. On one side of the spectrum, we'll see that GLP-1 users consume less of everything; on the other side, the reductions are very concentrated in only a few categories or even restricted to certain products in these categories.

Our four scenarios for the GLP-1 impact on the food & beverage industry



Source: ING Research

Key take-aways from our scenarios

Scenario	Narrative	2030 penetration (% of adult population)	Volume impact (% change in total calorie intake)	Category impact	Strategic takeaway for food & beverage companies
Shock & rebalance	Demand rises sharply in 2026-27, then stabilises as growth normalises.	8-12%	-1.25 to -1.75%	Concentrated in a few categories	Disproportionate volume pressure in most exposed categories; opportunities in nutrient-rich alternatives
Structural reset	GLP-1s become a mainstream health and prevention tool, supported by increased public spending on compensation.	15-20%	-2.5 to -3.5%	Widespread across categories	Companies must adapt to structurally lower demand; strong case for reformulation and nutrient density.
Overhyped	Initial excitement fades as long-term effects and willingness to stay on treatment disappoint.	3-5%	-0.25% to -0.5%	Concentrated in a few categories	Too small to materially reshape the sector. Limited need for product innovation and dedicated products.
Portion control era	Effects prove durable enough to matter, but not transformative enough to drive a prevention-led healthcare shift.	8-12%	-1.25 to -1.75%	Widespread across categories	Portion downsizing and reformulation become a central theme for a major part of the industry.

Source: ING Research

Calorie intake could drop by 2.5-3.5% in 2030 in a transformative scenario

In our most transformative scenario, increased use of GLP-1s has the potential to shave off 2.5-3.5% of total calorie intake in 2030 and higher percentages for the most affected categories. To make it more tangible, many GLP-1 users reduce their daily intake by 700 calories or more. That

is equivalent to two cheeseburgers at a fast food restaurant or three-quarters of a 200-gram bag of crisps. If the industry fails to adapt, that brings a risk of overcapacity in production.

In our overhyped scenario, the impact on demand remains very limited because usage of the drugs remains restricted to the most pressing cases and occasional use for weight loss. In this scenario, we'll see a reality check once initial success stories fade and scepticism rises.

The most likely outcome is that we see a combination of the scenarios across European countries. That's because countries: 1. have different public health systems, 2. face different levels of constraint on public health budgets, and 3. cope with varying levels of Type 2 diabetes and obesity prevalence. It's clear that things are changing within national health authorities; the NHS in England approved the use of Mounjaro® for certain patients in 2025, the HAS in France now allows general practitioners to prescribe the drugs, and Dutch authorities are considering adding more weight-loss drugs to the basic health package. But authorities are proceeding with caution because costs precede benefits. For European food and beverage makers, keeping track of such decisions will help to understand where the market is headed.

Food and beverage companies will adapt to offset any impact

Unlike many recent supply chain shocks, companies can prepare for the increasing adoption of GLP-1s. We recommend considering the following:

- **Product.** Product innovation, including reformulation, smaller portions and premiumisation, appears as a key response. Corporates including PepsiCo, Mondelez, Hershey and Danone have all highlighted in recent earnings calls that innovation is an important part of their adaptation strategy. They also like to frame it as an opportunity to offer new products, including high-protein and fibre-enriched versions. In the mid to long-term, increasing a company's presence in less affected products and categories (such as healthy snacking) through organic growth or M&A also fits this strategy.
- **Marketing.** When more people use GLP-1s, it makes sense for impacted companies to redesign marketing strategies. You would expect branded food and beverage makers to focus more on consumers who are not using GLP-1s, which is most of the population in any scenario. This includes consumers who could benefit from medication but don't have access to it. Then there is also a drive to re-engage with consumers as soon as they come off GLP-1s. After all, the temptations are still there, making it hard for consumers to sustain any changes in lifestyle and consumption habits. From a public health perspective, that is a bit of an inconvenient truth, but that's how markets work.
- **Geographic shift in sales markets.** By increasing focus on regions and countries with lower usage of GLP-1s. For European food and beverage makers that pursue volume growth, it will further increase the need to turn to growth markets in Asia, Africa and Latin America. However, GLP-1s will start making inroads in some of these markets too. Novo Nordisk's semaglutide patent is set to expire in several countries, including China, India and Brazil, and pharmaceutical companies are lining up to produce more affordable generic variations. So, for any company considering a shift in focus, it's a sensible thing to gain a better understanding of the dynamics around weight-loss drugs in these markets.

GLP-1s provide a solution at individual level, but they won't fix the underlying problem in the food system

GLP-1s are a technological fix for problems related to obesity and Type 2 diabetes, but these diseases are symptoms of a systemic problem. The underlying issue of a food system geared towards convenience and overconsumption remains. Rapid growth in GLP-1 usage shows they're an attractive solution at the individual level. However, it will require a lot more effort to get from a short-term solution at the individual level to a lasting solution at the public health level.

The impact on food and beverage demand would be more profound if policymakers grasped the GLP-1 opportunity by taking additional steps to steer the food system in a healthier direction by using a combination of prevention measures, increased taxation of unhealthy foods, subsidies for healthier alternatives and marketing restrictions. Thus far, we don't see European policymakers seizing that opportunity, also because such measures are not very popular and often viewed as overprotective.

Meanwhile, for food and beverage makers, a gradual adaptation strategy will likely be sufficient for 2026 and 2027. Nevertheless, it's wise to consider more extreme scenarios to avoid surprises a few years from now.

Author

Thijs Geijer

Senior Sector Economist, Food & Agri
thijs.geijer@ing.com

Diederik Stadig

Senior Economist, Healthcare & Technology
diederik.stadig@ing.com

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